SAMPLE ANSWER · STROKE · CASE 1

Sample 7-minute ISBAR: Ivy

Ivy Papadopoulos · 81-year-old woman · Acute stroke unit, public hospital · about 1033 spoken words (≈ 7.4 min at 140 words per minute)

I Introduction

53 words

"Hi, I'm the occupational therapy student who has just completed Ivy Papadopoulos's pre-discharge kitchen assessment. Ivy is an 81-year-old woman, eight days after a right hemisphere ischaemic stroke. I need to discuss her urgently, because she is due to go home alone by taxi at 1 pm and I have significant safety concerns."

S Situation

129 words

"Ivy is medically fit for discharge, and physiotherapy has cleared her to mobilise with a four-wheeled walker. During a tea-making task, however, she consistently missed things on her left. She clipped the left side of the door frame twice, turned on the back left hotplate when her saucepan was on the front right, and walked away to the fridge. She couldn't find the sugar on the left of the tray and read only the right-hand column of a shopping list.

She also told me there is 'nothing wrong with that side', and she plans to drive to the shops tomorrow morning. Her son has booked the taxi and the nurse in charge needs the bed for an ED patient, so I'd like your help before this discharge goes ahead."

B Background

150 words

"Ivy was admitted eight days ago and has mild weakness in her left arm and leg. Before the stroke she was independent, went to church every Sunday and drove herself to the shops. She is widowed and lives alone in a single-storey house. Her son Nick is her next of kin and lives in Toowoomba, about two hours away.

The medical team documented a brief cognitive screen two days ago as 'within normal range'.

There are several gaps I'd want to clarify rather than assume. I couldn't find documentation that Ivy has had driving advice since her stroke, whether a home visit has been considered, or what supports are arranged after discharge. I don't know whether anyone will check on her tonight, how she manages her medications, or whether ward staff have noticed similar difficulties. It's also unclear whether the medical team saw any left-sided problems during the cognitive screen."

A Assessment

339 words

"My main concern is that Ivy's presentation may indicate inattention to her left side, together with reduced awareness of her difficulties. That pattern is well recognised after right hemisphere stroke. I can't diagnose it, but it was consistent across walking, the stove, the tray and reading.

The most immediate consequence is safety at home alone. Ivy uses a saucepan on the stove at home. Today she turned on the wrong hotplate, walked away, and didn't recognise the error when I pointed it out. That is a fire and burns risk. Clipping the door frame also suggests a falls risk, particularly at night with nobody there.

Her reduced awareness matters as much as the inattention itself. Because she believes nothing is wrong, she's unlikely to use strategies like scanning to the left, and she may not ask for help.

The second immediate concern is driving. She is only eight days after a stroke, and with what I observed, driving tomorrow could put her and other road users at serious risk. Austroads' Assessing Fitness to Drive sets out a non-driving period after stroke, and fitness to drive needs medical review, not just her own judgement.

I also want to be clear about the reassuring findings. Physiotherapy has cleared her mobility, but that tells us she can walk safely in a ward corridor, not whether she attends to her left side during a complex task like cooking. A normal brief cognitive screen doesn't rule this out either, because brief screens often miss inattention that only shows up in everyday tasks. And Ivy being bright and chatty tells us about her conversation, not her performance.

So my priorities are: immediately, I don't think it's safe for Ivy to go home alone today without further assessment, and she shouldn't drive. Later priorities are a formal assessment of the left-sided inattention, a home assessment and support planning. As a student, I'm not determining her capacity or deciding her discharge, but I think this warrants urgent review by you and the medical team before she leaves."

R Recommendation

362 words

"First, I'd like your help to ask the nurse in charge and the treating team to put the discharge and the 1 pm taxi on hold until my findings are reviewed. I understand the bed pressure, but discharging her today could easily lead to a burn, a fall or a readmission. I'd explain this to Ivy respectfully, acknowledging how much she wants to get home.

Second, I'd escalate to the medical team today. I'd ask them to review my observations alongside their cognitive screen, give Ivy clear driving advice in line with Austroads, and document it.

Then I'd ask for MDT input. Nursing staff could tell us whether they've noticed left-sided difficulties on the ward, such as food left on the left of her meal tray. Physiotherapy could review her walker use in doorways and outdoors with this in mind. Social work could contact Nick, with Ivy's consent, about the plan, supports and options such as further inpatient rehabilitation or a supported discharge.

For OT, the principal short-term assessment I'd recommend is the Catherine Bergego Scale, using the Kessler Foundation Neglect Assessment Process. It rates left-sided inattention during real activities like eating, dressing and moving around, so it gives functional evidence rather than a screening score. I'd follow that with a home visit, ideally with Nick present.

For the longer term, I'd use the Canadian Occupational Performance Measure, so Ivy's own goals, which sound like getting home, cooking, getting to church and seeing her grandchildren, drive her rehabilitation and we can measure change.

Intervention could include visual scanning training and cueing strategies such as a coloured anchor line on her left, which fits the Stroke Foundation Living Clinical Guidelines. At home she may need stove safety measures such as an isolation switch or an automatic-shut-off kettle, plus supports like meal delivery or online shopping. Because she won't be driving for now, I'd help her plan other ways to get to the shops and to church, perhaps with her church friends or community transport, so she stays connected.

> I'll document today's observations clearly and hand over to the team, then follow up with you after the medical review to agree on next steps."

Coaching notes *not spoken*

The immediate priority

Ivy shows a consistent cluster of left-sided inattention with reduced awareness, and she plans to drive tomorrow. Discharge home alone today is unsafe without further assessment, and driving needs medical advice.

Distractors in this case

- **Physiotherapy clearance with a walker** — shows safe walking on the ward, not attention to the left during complex tasks.
- **Normal brief cognitive screen** — brief screens can miss inattention that only appears in functional tasks.
- **Bright, chatty and talking about church and grandchildren** — sociable conversation hides performance problems; observe, don't infer.
- **Son's taxi booking and the bed pressure** — real pressures, but they don't change the risk. Name them and escalate anyway.

Missing information to clarify

- Whether any driving advice has been given or documented
- Who, if anyone, will check on Ivy after discharge; community supports
- Home layout, stove type and whether a home visit has been considered
- Nursing observations of left-sided difficulties on the ward
- How she manages her medications

Useful references

- Stroke Foundation, *Living Clinical Guidelines for Stroke Management* (neglect and perceptual impairment; discharge planning)
- Austroads, *Assessing Fitness to Drive* (stroke and neurological conditions)
- Azouvi P. et al. on the Catherine Bergego Scale; Chen P. et al. on the Kessler Foundation Neglect Assessment Process (KF-NAP)

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